

1*. The complications of hemangiomas are the following with exception:

- A. exulceration
- B. infection
- C. malignization**
- D. hemorrhage
- E. thrombocytopenia (Kassabach-Meritt)

2*. In type 1 esophageal atresia:

- A. both esophageal segments have a communication with trachea;
- B. the abdomen is progressively distending due to air accumulation in the stomach;
- C. the esophageal segments have no link to trachea;**
- D. the introduction of a nasogastric tube stops nausea;
- E. the feeding of newborn will be done in little amounts every 2 hours

3*. The preferred surgical technique in the hiatal hernia is:

- A. gastropexia
- B. fundic plication of Nissen;**
- C. subtotal esofagoplasty
- D. subtotal gastroectomy
- E. no surgical treatment is necessary, only medical

4*. The typical contrast imaging in duodenal atresia shows:

- A. narrowing and elongation of the pyloric segment
- B. 'corkscrew' image
- C. 'double bubble' in the superior abdomen**
- D. A+B
- E. A+C

5*. In type 1 interstitial atresia, one may notice:

- A. the continuity of the sero-muscular wall with a membrane blocking the lumen**
- B. between the closed segments of the gut there is a fibrous cord, the mesenter is intact or has a 'V' interruption;
- C. the closed interstitial segments are completely separated, the mesenteric defect is in 'V'
- D. multiple completely separated atretic areas, the 'V' shaped mesenteric defects take the aspect of a 'pagoda'
- E. none of the above mentioned aspects

6*. The tumour in intussusception:

- A. is composed by 2 cylinders
- B. is composed by 5 cylinders
- C. is composed by 4 cylinders
- D. has a intussuscepted cylinder, one is recipient and one intermediary;**
- E. the external cylinder is formed on behalf of the penetrating or receiving loop

7*. The most frequent affected region in the congenital megacolon is:

- A. cecal
- B. ileal
- C. recto-sigmoidian**
- D. jejunal
- E. anal

8*. The X-ray with barium in intestinal malrotation with volvulus shows:

- A. 'double bubble' image
- B. 'corkscrew' image**
- C. opacification of the whole colic frame
- D. the retention of the contrast in the volvulated intestine
- E. the presence of the dye above and below the volvulated intestine

9*. The primitive peritonitis are most frequent

- A. staphylococcus
- B. pneumococcus**
- C. gonococcus
- D. meningococcus
- E. streptococcus

10*. The most frequent localization of the hydatid cyst is:

- A. in the lungs
- B. in the liver**
- C. in bones
- D. in the spleen
- E. in muscles

11*. The sacrococcygeal teratoma has his origin in:

- A. ectoderma
- B. endoderma
- C. mesoderma
- D. ectoderma, endoderma and mesoderma**
- E. none of above mentioned strucutres

12*. The curative treatment of the testicular torsion consists of:

- A. local administration of non-steroidal anti inflammatory drugs
- B. ice bag application in the first 6 hours
- C. emergency surgical intervention**
- D. antibiotic administration in high dosage
- E. sedation

51. Cephalhematoma:

- A. is a subperiostic hemorrhagic collection;**
- B. is a subcutaneous hemorrhagic collection;
- C. is more frequent in the frontal area and has fluctuence;
- D. has a slow evolution toward resorbtion in 2-4 weeks with unaesthetic callus;**
- E. needs correction of hypo-prothrombinemia (vitamin K, C, Ca) and evacuation after few days**

52. Obstetical cranial fractures:

- A. are often seen due to cartilaginous sutures that allows cranial deformity;
- B. are most frequent linear fractures;**
- C. are most frequent depressed fracutres;
- D. are sometimes accompanied by cephalhaematoma;**
- E. depressed skull fractures beyond 5-6mm requires surgical correction because they can cause neurological disorders**

53. Vrolich Disease (osteogenesis imperfect- the congenital precocious form):

A. is autosomal recessive transmitted;

B. starts and evolves in the intrauterine life, the child presents with multiple fractures at birth;

C. is autosomal dominant transmitted;

D. it is known as the 'brittle bone disease'

E. has a bad prognosis

54. The structural classification of lymphangioma is:

A. plain lymphangioma

B. simple lymphangioma

C. cavernus lymphangioma

D. mesenteric lymphadenitis

E. cystic lymphangioma or hygroma

55. The cyst and fistulas derives from the second brachial arch;

A. are more frequent than those derivate from the first brachial arch;

B. may be uni- or bilateral;

C. are located on the anterior margin of sternocleidomastoidian muscle;

D. are located on the anterior margin of deltoid muscle

E. can be tolerated long period, but get infected by time

56. Meningoencephaloceles:

A. is more frequent than myelomeningoceles;

B. is located mostly lumbar;

C. is produced on the median, frontonasal and occipital line;

D. occurs due to the suture defect of the cranial bones;

E. one or more vertebrae have the medullar channel open.

57. Esophageal atresia:

A. has absent esophageal lumen in his medial third;

B. is the most frequent malformation of the urinary system;

C. is incompatible with life;

D. sex ration is equal;

E. the lesion occurs at the embryo between 24-27 weeks

58. Congenital diaphragmatic left hernia presents with:

A. enlarged peritoneal cavity;

B. depressed, hypoplastic and pushed left lung toward the pleural dome;

C. the lung herniates in the abdomen;

D. the heart and mediastinum are deviated to the right;

E. the abdominal viscera protrude in pleural cavity

59. Hiatal hernia:

A. appears through the Morgagni foramen;

B. appears though the Bochdalek foramen;

C. appears through esophageal hiatus;

D. associated with hypertrophic pyloric forms Roviralta syndrome;

E. appears through foramen ovale.

60. The differential diagnosis of hypertrophic pyloric stenosis consist of:

- A. feeding mistakes;**
- B. regurgitation**
- C. milk intolerance**
- D. ano-rectal malformation;
- E. meconium ileus.

61. The major form of omphalocele consists of:

- A. Parietal defect less than 2cm;
- B. parietal defect less than 5cm;
- C. hernia of the intestine and a part of liver;**
- D. the dimensions of omphalocele causes difficult expulsion of the fetus and membrane rupture risk;**
- E. edema and retraction of the intestinal loop

62. Laparoschisis presents with:

- A. right paraumbilical parietal defect 2-3 cm, through which herniates the small and large intestine;**
- B. absent umbilical cord;
- C. malrotation of small intestine through lack of adhesion, intestinal loops are edematous, enlarged and grey due to the contact with amniotic fluid;**
- D. shorter intestine (up to 1/3 of normal length)**
- E. very rare associated with prematurity.

63. Omphaloenteric fistula:

- A. represent the persistence of internal portion of omphaloenteric duct**
- B. is covered with secretor digestive mucosa that produce a serous secretion**
- C. exploration is made with a metallic stilet or radiologic examination with contrast in profile exposure;
- D. the treatment is surgical and consist of fistula excision**
- E. surgery is not necessary

64. Meckel's diverticulum:

- A. is a rest of the omphalomesenteric duct present in 1.5% of population;**
- B. usually asymptomatic is diagnosed by the associated complications;**
- C. present gastric mucosal islets (duodenal, jejunal, colic or pancreatic mucosa) and in the interior has intestinal mucosa;**
- D. has a dimension between 1-8m;
- E. the lumen is larger than the colon

65. In the umbilical urachal fistula:

- A. there is a communication between umbilicus and gallbladder;
- B. one can notice clear fluid (urine) at the umbilical button;**
- C. one can notice intestinal fluid at the umbilical button
- D. urine examination demonstrate urinary infection of the bladder**
- E. surgical treatment consist of double ligation and resection of the fistula**

66. The duodenal malformations could associate one or more of the following:

- A. mongolism (Down Syndrome) in 30% of cases;**
- B. cardiac malformations;**
- C. venous malformations
- D. small intestine malformations**
- E. prematurity**

67. Intussusception:

- A. represent introduction of a portion of small intestine through a mesenteric defect;
- B. represent the protrusion of a suprajacent segment into subjacent segment of small intestine**
- C. result in mechanical intestinal obstruction;**
- D. it is more frequent in newborns;
- E. it is more frequent in infants.**

68. Hirschsprung disease present with:

- A. the absence of intestinal transit;**
- B. progressive abdominal distension;**
- C. bulimia;
- D. vomiting-initial food, then bilious and at the end faces;**
- E. bad smelling soft stools and paradoxical diarrhea**

69. In anal stenosis:

- A. anal orifice is normally located;**
- B. anal orifice is ectopic;
- C. anal channel is incompletely block by a mucus diaphragm;**
- D. anal channel is completely blocked by a mucus diaphragm;
- E. the feces has a thin pen like aspect.**

70. Intestinal malrotation is present in:

- A. congenital inguinal hernia;
- B. omphalocele;**
- C. laparoschisis;**
- D. duodenal stenosis or intestinal atresia;**
- E. hypertrophic pyloric stenosis

71. Mesenteric lymphadenitis:

- A. is the most frequent cause of abdominal pain in infants;
- B. is represent by the inflammation of mesenteric ganglions from ileocolic region;**
- C. is represent by the inflammation of retroperitoneal ganglions;
- D. etiology is represented by viruses and bacteria**
- E. the involved ganglions are hypertrophied and associates fluid peritoneal reaction

72. Clinical picture in mesenteric lymphadenitis is based on:

- A. periumbilical pain in crisis that come over and over;**
- B. fever 39 degree Celsius with good general status;**
- C. with or without vomiting**
- D. polyuria and polydipsia
- E. localized pain in the ureteral points exactly identified by patient

73. Treatment of primitive peritonitis consists of:

- A. large dosis of Oxacilin (3-6 g/24h);
- B. biopsy of mesenteric ganglions;
- C. large dosis of Penicilin (3-6mil/24h)**
- D. if it is doubt surgery is indicated;**
- E. if neglected with cystic peritonitis drainage and antibiotics are associated.**

74. Simple meconium ileus:

- A. is localized in the proximal portion of the small bowel;
- B. is localized in the distal portion of the small bowel;**
- C. the proximal segment is dilated (7-10cm) with muscular hypertrophy + the meconium is viscous and adherent;**
- D. distal ileon has a reduced calibre with storage of meconium like pearls;**
- E. large bowel has a microcolon aspect contains mucus and meconium**

75. Differential diagnosis for meconium ileus is made with:

- A. duodenal atresia;
- B. megacolon;**
- C. mesenteric lymphadenitis;
- D. left small colon;**
- E. colic atresia**

76. Inguinal hernia:

- A. the most frequent cases are diagnosed at premature;**
- B. is more frequent in girls;
- C. is never bilateral;
- D. is more frequent bilateral;
- E. is more frequent in boys**

77. Inguinal hernia in the school child:

- A. is visible only at the effort and coughing (excepting the large incoercible hernias);**
- B. results from failure to obliterate of the umbilical ring;
- C. is always bilateral;
- D. at physical examination one can observe a large external orifice of the inguinal channel at the affected side;**
- E. for all male cases is mandatory to check the presence of the testicles in the scrotum**

78. Clinical picture in pulmonary hydatid cyst consist of:

- A. coughing-initial is iritative and than is mucopurulent;**
- B. dyspneea-always;
- C. hemoptisia-repetative;**
- D. allergic skin modifications (urticaria) to some patients;**
- E. intercostal pain if a pleural reaction is present**

79. Nephroblastoma is clinically manifested as:

- A. variable dimension tumor with lumbar contact, ferm, easy to mobilize;**
- B. vague pain, sometimes irradiating on the tight, with limping;**
- C. sometimes macroscopic hematuria;**
- D. Blood hypertension due to renal artery compression;**
- E. diarrhea

80. Ureteral anomalies in calibre and shape include:

- A. retrocaval ureter;
- B. ureteral diverticulum;**
- C. ureterocel;
- D. congenial ureteral stenosis**
- E. congenital megaureter**

81. Vesico-ureteral reflux

- A. is the flow back of the urine from bladder to superior urinary tract**
- B. is the flow back of the urine from urethra to the bladder;
- C. is secondary to Hirschsprung disease;
- D. Is diagnosed through mictional ureterocystographie**
- E. requires uretrovesical antireflux reimplantation**

82. Acute scrotum includes the following:

- A. testicle torsion;**
- B. Morgagni testicular torsion;**
- C. retractile testicle;
- D. testicular and scrotal trauma;**
- E. acute scrotal edema**

83. Differential diagnosis for adnexial torsion in girls less than 5 years is made with:

- A. acute appendicitis;**
- B. renal colic;**
- C. orhiepididimitis;
- D. rupture of the ovarian cyst;**
- E. ectopic extra uterine pregnancy

84. Stage 1 varicocele:

- A. is evident through inspection;
- B. appears only in clinostatism;
- C. is evident only by palpation;**
- D. disappears only in clinostatism;
- E. do not implies macroscopic testicle modifications**

85. Pericardial tamponade:

- A. results from accumulation of air in the pericardial cavity;
- B. follows thoracic trauma with penetrated wound;**
- C. results from accumulation of blood in the pericardial cavity;**
- D. always has fatal evolution
- E. treatment is pericardiocentesis**

86. Cerebral concussion:

- A. results after minor cranio-cerebral trauma;
- B. lack of consciousness is transitory and for less than 10 min;**
- C. is determinate by perturbation of the cerebral functions due to an organic lesion;
- D. is spontaneous reversible;**
- E. it is possible to occur epilepsy.**

87. Hypertrophic pyloric stenosis is clinically manifested through:

A. 2-3 weeks free interval from birth followed by bilious vomiting;

B. 2-3 weeks free interval from birth followed by white projectile vomiting;

C. as the stenosis progress the vomiting becomes bilious;

D. immediately after vomiting the child wants to eat again;

E. as the disease progress the vomiting becomes less frequent but more abundant

88. The testicular tumors treatment includes:

A. surgical treatment;

B. chemotherapy

C. cobaltotherapy

D. immunotherapy

E. antibiotherapy